

often not discovered until this point in labor, and immediate action is then needed to deliver the rest of the body safely. The nurse will call for extra help. You will be asked to stop pushing and to draw your knees close to your chest to allow more room in the pelvis. An episiotomy may be needed to allow the obstetrician to perform internal maneuvers to help release your baby's shoulders. Shoulder dystocia is more likely to occur where the baby is particularly large, where the mother has a small pelvis, or in women with diabetes or who are obese. If you have had a shoulder dystocia in a previous pregnancy, there is an increased risk that you will have another shoulder dystocia. You may be offered a Cesarean section to prevent this from recurring.

Episiotomies

An episiotomy is a cut made between your vagina and anus to aid the delivery. Around 30 years ago, episiotomies were routine since it was assumed that a cut prevented worse tears. This has been shown since not to be the case, and today, an episiotomy is only done by a doctor to aid a forceps delivery or in an emergency—or example, if the baby is in distress. Before doing the procedure, the doctor should explain why they think it is necessary and should obtain your verbal consent.

Before you're cut, you'll have a local anesthetic to numb the area. The cut is usually done at an angle between the vagina and the rectum and will be stitched after the birth. The midwife or doctor will put together all the layers of the episiotomy, including the muscles, posterior vaginal wall, and skin. If you feel discomfort, the stitches can be snipped or removed. You may be given oral analgesics for pain relief after you've been stitched, and anti-inflammatory medication may be recommended. Ice packs or a maternity cooling gel pad can ease pain and swelling, and bathing can be soothing.

Perineal tears

Some women tear naturally during the delivery of their baby and this tends to be more common in a first labor. Spontaneous tears are classified by their severity and the tissue layers involved. A first-degree tear involves the skin layer only; a second-degree tear involves the skin and muscle-tissue layers; a third-degree tear involves skin, muscle, and the anal sphincter; a fourth-degree tear involves the anal sphincter and the rectum. A first-degree tear usually doesn't need stitches, but second-, third-, and fourth-degree tears do. Second-degree tears are the most common; only a small number of women sustain third- or fourth-degree tears.

BIRTH STORY: WATER BIRTH

Becky is a 22-year-old woman in her first pregnancy. She'd had no problems and at her 36 week appointment, she discussed her birth plan and told her doctor that she wanted a water birth at the local birth center.

Becky's birth story: By the time I was two days overdue I was having irregular contractions. I'd had a bloody show and had a backache. The next day I woke at 6:30 am with regular contractions. I coped using breathing techniques, stayed active, and used a birthing ball. When my contractions were every 5 minutes and lasting a minute, I called the birth center since I felt I needed to use the pool. They said they would get it ready.

At 11:35 am, my partner and I reached the center and met the doctor. She felt my contractions, listened to the baby's heartbeat and did a vaginal examination. She said I was in established labor since I was 5 cm dilated, my cervix was 50 percent effaced, and the head was descending. In the pool, I was able to move around and change position. The warm water eased my backache and contractions. I felt calmer and started to relax. I found kneeling in the water